REPORT TO OUR DONORS

Building the evidence a MED13 treatment needs

When we started in October 2025, the published literature described about 28 people with MED13-related disorder.[4] There was no longitudinal natural history data, no standard of care and no clinical trial. A Broad Institute model estimates that new disease-causing MED13 variants arise in about 1 in 82,000 births, which suggests many more people are undiagnosed.[3]

Our patient-governed, longitudinal natural history study and biorepository complements Simons Searchlight, which began collecting MED13 data before we were founded. We use it to advocate for research toward treatments and to convene scientists, patients, physicians and the community.

Where we stand

Numbered sources at the foot of the page

FAMILIES AND DATA

NATURAL HISTORY STUDY

9 / 200+

enrolled from 6 countries, of more than 200 people diagnosed worldwide[1, 2]

BIOBANK CONSENT

9 / 9

families consented to biobanking and future research contact[1]

OUTCOME MEASURES

3 / 9

participants with a scored measure of adaptive function, sleep or quality of life[1]

RESEARCH AND FUNDING

DISEASE MODELS: COMMITTED VS DEPLOYED CAPITAL

$22,000 deployed of $56,000 committed

Deployed **$22,000** · Committed, not yet deployed **$34,000**

4 models at about $14,000 each (blood draw, multiomics, iPSCs); 1 in progress[6, 7]

SPENDING ON RESEARCH

$22,800 / $38,360

of 2026 expenses went to research grants and the biorepository[6]

ROADMAP

6 / 22

milestones done or active, with 5 in progress and 2 next

Why four disease models. Published research suggests MED13 variants act in at least two ways: some lower the amount of MED13 protein, and others change how it is broken down. A treatment that works for one may not work for the other, so we are funding a model for each of four variant classes.[4, 5]

Progress on our roadmap

Of 22 milestones, 6 are done or active, 5 are in progress and 2 are next.

1. **Foundation Building** 2025 to 2027 — ●●●●
   Active: Registry and biorepository, advisory board, family network
2. **Research Infrastructure** 2026 to 2028 — ●●◐◐◌◌
   Active: Research grants; Done: First scientific symposium. In progress: outcome measures, variant analysis. Next: disease concept model, Mini Meeting of the MEDs in Denver
3. **Preclinical Development** 2028 to 2030 — ◐○○○○
   In progress, ahead of schedule: models of disease for assays
4. **Trial Readiness** 2030 to 2032 — ◐◐○○
   In progress: natural history study, biomarker validation
5. **Clinical Trials** 2033 onward — ○○○

● Done/active ◐ In progress ◌ Next ○ Not started

Next steps

Oct 2026	Prioritize the next 3 of 4 disease models with our working group and Scientific and Clinical Advisory Board.
Fall 2026	Pilot multiomics with imyoo on biorepository samples under an approved IRB protocol.
Fall 2026	Help each enrolled family complete three core measures: adaptive function, sleep and quality of life.
Dec 2026	Meet the MED12 and MED13L communities at the Mini Meeting of the MEDs in Denver.
Fall 2026	File the foundation's own application for 501(c)(3) recognition.

Move a number

Donors: fund the disease concept model ($10,000) and the Mini Meeting of the MEDs in Denver, December 2026 ($7,000).
Families: enroll in the natural history study; each enrollment tells us more about the biology of MED13.
Enrolled families: complete your outcome surveys on adaptive function, sleep and quality of life.
Researchers: request de-identified data through COMBINEDBrain under a data use agreement.

Give at med13.org

amy@med13.org

Sources

1. MED13 Foundation natural history study on the COMBINEDBrain Matrix platform, data cut of 10 August 2026.
2. MED13 Foundation estimate of people diagnosed worldwide, as published on med13.org.
3. GenIE, Broad Institute: MED13, estimated genetic incidence of de novo variation, 1.216 per 100,000 births; a modeled estimate, not a count of diagnosed people. Accessed 12 October 2026. genie.broadinstitute.org
4. MED13 Foundation review of published case reports, 2026. First description: Snijders Blok L, et al. Human Genetics. 2018;137(5):375-388. doi.org/10.1007/s00439-018-1887-y
5. Davis MA, et al. The SCF-Fbw7 ubiquitin ligase degrades MED13 and MED13L. Genes and Development. 2013;27(2):151-156. doi.org/10.1101/gad.207720.112
6. MED13 Foundation financial records, 1 January to 10 October 2026, unaudited. Funds are held by fiscal sponsor Rare Village Foundation.
7. MED13 Foundation commitment to fund four patient-derived disease models at about $14,000 each (blood draw, multiomics and iPSC generation), October 2026. Deployed: research grant paid in 2026.

MED13 Foundation is a California nonprofit public benefit corporation preparing its own application for 501(c)(3) recognition. It operates as a fiscally sponsored project of Rare Village Foundation (EIN 83-4699994), a 501(c)(3) public charity, and gifts made through Rare Village Foundation are tax-deductible to the extent allowed by law. Study groups smaller than 20 are reported as counts, not percentages. Nothing in this report should guide any individual's medical care.